

Mental Illness Terms and Hermeneutic Hijacking¹

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*Yet it is hardly possible to take up one's residence in the kingdom of the ill unprejudiced
by the lurid metaphors with which it has been landscaped.*

-Illness as Metaphor, Susan Sontag (1977)

Abstract

Nonliteral uses of mental illness terms abound. Common examples include criticizing a person's mood swings by saying "you're bipolar", hyperbolically exclaiming that a particularly difficult college exam "gave me PTSD", or responding to a compliment about the cleanliness of one's home by saying "I just have OCD." There has been some pushback in recent years, both socially and within the philosophical literature, against using mental illness terms nonliterally to imply something negative about the subject. The focus so far has been on the pejorative nature of the utterances. Nonliteral uses which imply something positive about the subject have all but gone ignored. One might think that these positive uses are innocent, perhaps even helpful in combatting stigma. On the contrary, as I argue in this paper, all nonliteral uses of mental illness terms are harmful, and they are harmful for the same reason. I argue that they contribute to what I call

¹ I would like to first thank two anonymous referees and the editors for their helpful feedback in shaping this paper. I especially would like to thank Robin Jeshion, who has seen every version of this paper from the very beginning; this paper would not exist without her. Moreover, thank you to Mark Schroeder for his unending support and invaluable advice. I also want to extend my gratitude to others who have provided feedback on various versions of this paper, including (among others I have surely and apologetically forgotten to mention here) Ethan Higginbotham, Kayleigh Rodgers, Rebecca Carlson, Shane Ward, Laura Nicoară, Brian Haas, and David Clark.

hermeneutic hijacking. Hermeneutic hijacking occurs when a term's literal meaning is eclipsed by a nonliteral usage in a way that prevents the original term from functioning as it should. Though people have recognized the harm of pejoration, they have overlooked the way that nonliteral uses of mental illness terms, even absent pejoration, cause hermeneutic injustice.

1. Introduction

In an episode of his talk show in April 2020, Stephen Colbert made the following joke in reaction to a recent interview with Dr. Anthony Fauci regarding the COVID-19 pandemic: “Dr. Fauci wants compulsive handwashing to be the new normal, which is great news for anyone with obsessive-compulsive disorder. Congratulations! You now have obsessive-compulsive *order*! Also being recommended by the CDC: counting the tiles on your kitchen floor and making sure your shoes all face north” ([The Late Show with Stephen Colbert, 2020, time code: 2:14-2:36](#)). Similar jokes can be found on all forms of social media, relying on the cultural narrative that a person with obsessive-compulsive disorder (OCD) is best characterized as the person who washes their hands an obnoxious amount, suggesting that those with OCD are experts at handling the global pandemic. The Wall Street Journal has even published an opinion piece entitled “We All Need OCD Now” (Elias Aboujaoude, 2020).

Although jokes about OCD have been especially prevalent in recent years, jokes that gesture at some mental illness or another are not new.² It is quite common to hear people use psychiatric terms and phrases nonliterally. Other common examples include criticizing a person's mood swings with the phrase “you're bipolar” or hyperbolically

² I use the terms ‘mental illness’ and ‘mental disorder’ interchangeably in this paper.

exclaiming that a particularly difficult college exam “gave me PTSD.” When people say these things, they often are not intending to make any actual claims about whether they themselves or someone else has the mental illness that they are gesturing at, but instead mean to suggest that the person being referred to shares some characteristic with someone who they think typifies the term or phrase.

In recent years, there have been some efforts to push back against pejorative nonliteral uses of mental illness terms both socially and within the philosophical literature. Renée Jorgensen (2021), for example, has argued that derogatory metaphorical uses of terms like ‘schizo’ and ‘psychopath’ function as general pejoratives (like ‘asshole’ or ‘jerk’), and that they can wrong those who are mentally ill in at least two ways: directly, by illegitimately diminishing their credibility in a given conversation, and indirectly, by contributing to the stereotype that a mental illness makes one untrustworthy. Both within academia and beyond, a common assumption seems to be that it is the fact that these nonliteral uses are pejorative that explains the harm. One might think that nonpejorative uses are not harmful, or at least not harmful for the same reason. Some may even think that they are *helpful* in combatting mental illness stigma.

To feel the pull of this intuition, consider another case: the pejorative use of ‘gay’ commonly heard in the early 2000s. As evidenced by the multitude of social awareness campaigns, saying something is “gay” when one means “lame” or “bad” is harmful. It conveys the attitude that being gay is a bad thing, contributing to a culture of homophobia. At least in polite society, it is now unacceptable to metaphorically use the term ‘gay’ in a pejorative way. What about using ‘gay’ metaphorically in a positive or neutral way? Such usages are not as common, but they do exist, especially amongst young (primarily queer) people on social media. One might see the following comment on TikTok, intended as a

compliment: “I love your outfit—very gay.” Here, ‘gay’ is being used metaphorically to indicate that the speaker thinks the person they are talking about embodies a particular aesthetic that is prominent in online Queer culture; successful understanding depends on recognizing that the speaker thinks embodying such an aesthetic is cool. These sorts of metaphorical uses have not received the same pushback as their pejorative counterparts. At least at first glance, such uses seem relatively harmless. This gives us some reason to think that the pejorative metaphorical usage of ‘gay’ is harmful because it is pejorative and not because it is metaphorical.

One might think that mental illness metaphors are similar—if they are harmful, it is only (or primarily) insofar they are pejorative; neutral and positive mental illness metaphors are innocent. In this paper, I argue that this bifurcated treatment of mental illness metaphors is a mistake. Both pejorative and nonpejorative mental illness metaphors contribute to what I call *hermeneutic hijacking*. Hermeneutic hijacking occurs when a term’s literal meaning is eclipsed by a nonliteral usage in a way that prevents the original term from functioning as it should. In other words, nonliteral uses of mental illness terms make it extremely difficult, if not impossible, to use the terms literally. It is this loss that causes harm to those who are mentally ill. Though people have recognized the harm of pejoration, they have overlooked the way that mental illness metaphors, even absent pejoration, cause hermeneutic injustice. The account I provide here complements recent work in the literature on the ethics of metaphor by providing a linguistic mechanism which goes some way towards explaining the harm done by certain metaphors.³

³ Of particular relevance is the work of Rachel Elizabeth Fraser (2018), Lucienne Spencer and Havi Carel (2021), and Kaitlin Sibbald (2023), all of whom provide an account of how metaphors can contribute to hermeneutical injustice.

The paper proceeds as follows. In Section 1, I clarify which terms fit within the scope of the argument by giving a more detailed account of nonliteral uses of mental illness terms. In Section 2, I introduce the concept of hermeneutic hijacking and argue that it is a consequence of these nonliteral uses. In Section 3, I explain the harm imposed by the hermeneutic hijacking of mental illness terms. In Section 4, I consider how my analysis may extend to the metaphorical use of other terms.

Before I move on, I want to acknowledge that there are compelling and relevant criticisms of the diagnostic criteria for the mental illnesses I will be discussing in this paper, as well as of the state of psychiatry more broadly. Although I will be referring to current diagnostic practices as a standard on which we should rely, I do not wish to suggest that these practices are faultless. Although I use the term ‘mental illness’, I recognize that the DSM in its current state includes things that may better be classified as neurodivergences and things that ought not to be medicalized at all. The key feature of the category of terms I will be discussing is not that they are illnesses, but that they are ways of classifying people that are often highly central to people’s identities. Some readers may insist that some conditions and/or their diagnostic criteria, even used literally, are pejorative in nature and not merely by association.⁴ If this is the case, then the impact of losing the term will need to be weighed against the harm of maintaining a pejorative term. The points I make are consistent with the idea that our understanding of mental illnesses should continue to evolve in ways that accurately and ethically describe the world. Finally, it is important to note that receiving a diagnosis for a mental illness can open up to various kinds of epistemic injustice, and testimonial injustice in particular, even when the

⁴ Jen Foster’s (2023) article will especially be of interest to readers who have this concern.

diagnosis is aptly applied. I return to the way this fact relates hermeneutic hijacking in the sections below, but I do not intend hermeneutic hijacking to fully explain the ways in which receiving a diagnosis makes one vulnerable to epistemic harms.

2. Nonliteral Uses of Mental Illness Terms

Mental illness terms (MITs henceforth) convey the content that is given by diagnostic terms in the current edition of *The Diagnostic and Statistical Manual of Mental Disorders* (DSM-5, at the time of writing) or *International Classification of Diseases* (ICD-10-CM, at the time of writing). Literal uses of MITs are, at least paradigmatically, clinical uses, and are concerned with diagnosis, treatment, research, etc. Mental health professionals may also use MITs literally in ways that do not align perfectly with current diagnostic criteria in discussions about refining the term (for example, in taskforces created prior to new editions of the DSM). At least for now, I restrict my argument to terms and phrases from the DSM or ICD; I explore other terms and phrases in Section 4.

Return to Colbert's joke about OCD. When used literally, the term 'OCD' is an acronym for the mental health disorder obsessive-compulsive disorder that is identified in the DSM. If Colbert were speaking literally, he would be committed to the claim that the pandemic has made it the case that OCD is no longer dysfunctional but is instead advantageous. Colbert does not mean to say that, though. Instead, he is saying something like the following: the practices recommended by experts to combat the COVID-19 pandemic are so far from "normal" that everyone is acting like stereotypical portrayals of a person with OCD. Were someone to correct Colbert—perhaps by saying "what you are describing is not actually OCD"—he would simply respond that they had missed the point.

Nonliteral uses of MITs may not always fit neatly into a particular type of nonliteral speech. They are often similar to loose talk or metaphor, however, so I will use these as

examples. I intend my account to generalize to other forms of nonliteral speech, such as hyperbole.

Someone speaks loosely when they say something false in an effort to convey some set of information that is more effectively communicated by way of the false sentence than a sentence that is literally true. Dan Sperber and Deirdre Wilson (1985: 162) describe the phenomenon technically:

The speaker wants to communicate to her hearer a certain set of propositions $P_1 \dots P_n$. They are all quite easily derivable as logical or contextual implications of a proposition Q whose truth she does not believe and does not want to guarantee. The best way of conveying this information may be to express the single proposition Q as long as the hearer has some way of selecting those of its logical and contextual implications that the speaker intends to convey, and ignoring the others.

For example, imagine that my brother Ryan is coming to visit me and his plane lands at 11:50 am. I am at lunch with a group of friends and need to make clear why I am leaving early. If I say, “Ryan’s flight lands at noon,” then what I have said will be false. However, I effectively convey the information that is important: that I need to leave soon, that I am not being rude by doing so, etc. This is more effective than saying “Ryan’s flight lands at 11:50.” The specificity of the latter sentence is odd in casual conversation, and very likely conveys more information than I intend to convey.

Now imagine that someone says of a disliked coworker, “It is so hard to work with him—he’s bipolar!” Assume, for the moment, that the coworker does not meet the diagnostic criteria for bipolar disorder, making the phrase “he’s bipolar” false. In cases that I am interested in, this will not matter; the speaker is not interested in whether or

not their coworker actually has bipolar disorder. Context will clue the hearer in to what the speaker intends to convey: that the coworker is highly temperamental, that his moods negatively affect those around him, etc. In cases like this, the speaker is using the term ‘bipolar’ loosely.

MITs are also used metaphorically. Metaphors allow speakers to convey complex thoughts in very little words, relying on the hearer to pick out the salient features between the subject of the sentence and the thing they are likened to.⁵ I often joke and say, “My pony is a big goofy dog,” especially to people who are unfamiliar with horses. What I am trying to communicate is that my pony is very friendly, lovable, and gentle. I am using a term to highlight salient features that the predicate (in this case, ‘big goofy dog’) has that apply and are apt to describe the subject (‘my pony’). When a speaker uses an MIT metaphorically, they are attempting to highlight salient features that the subject of their sentence and a person who they take to typify the relevant disorder or symptom have in common. If someone refers to their coworker as “bipolar” metaphorically, the comparison will highlight the coworker’s supposedly extreme mood fluctuations and their negative effect on others.⁶

Importantly, a speaker does not need to be aware that they are speaking nonliterally. Many metaphors are so common that some speakers may believe them to be literally true when using them. In fact, a metaphor can become so common that the once-metaphorical meaning becomes conventionalized. This is part of the problem, as I will

⁵ There are competing accounts of how metaphors function, any of which should be compatible with my description here. See for example: Elisabeth Camp (2006); Donald Davidson (1984); David Hills (1997); Ernie Lepore and Martin Stone (2010).

⁶ It may be the case that the subject in fact has the mental illness that the MIT refers to, but this does not mean that the term is not being used nonliterally. What matters is that the speaker is using the term to convey the stereotypes associated with the nonliteral meaning and not the mental illness itself.

describe in the following sections. A speaker may use an MIT while believing that they are speaking truthfully; they may believe that to be bipolar (for example) just *is* to be moody and unpredictable. In at least some instances, speakers will still be using the term nonliterally. The speaker's attitude and intention about how they are using the word does not fully determine whether their usage is nonliteral, nor does it bear on whether their usage contributes to hermeneutical hijacking.

I have sketched examples of loose talk and metaphor. Nonliteral MITs may be either of these or something in between, but the relevant feature is that they will be used to say something other than what they literally mean. Nonliteral uses of MITs may be true. While speaker intentions matter somewhat, the speaker may be wrong about whether their use of the term is nonliteral. All of this will be helpful in detailing how hermeneutic hijacking occurs and why, in the case of MITs, it is harmful, as I describe in the next two sections.

3. Hermeneutic Hijacking

Hermeneutic hijacking occurs when the meaning of a particular term when used nonliterally eclipses the term's meaning when used literally.⁷ Hermeneutic hijacking is characterized by a linguistic aspect and a social aspect. Linguistically, there is a semantic shift; the nonliteral meaning replaces the literal meaning. This is not necessarily a bad

⁷ Hermeneutic hijacking as I will go on to describe it is similar to, but distinct from, other recently introduced terminology. Of particular relevance are the concepts of hermeneutic pruning and dilution introduced by Kaitlin Sibbald (2023) and hermeneutic sabotage introduced by Han Edgoose (2024). Like the project I engage in here, both Sibbald and Edgoose describe mechanisms by which marginalized groups lose the ability to use important terms to communicate to those outside of the group. The mechanism I describe differs in important ways. Unlike hermeneutic sabotage, hermeneutic hijacking is not necessarily intentional nor is it caused solely by members of dominant social groups. Further, whereas Edgoose and Sibbald focus primarily on the ways vulnerable populations may lose the ability to communicate to others, I am equally concerned with cases where individuals do not have access to hermeneutic resources to recognize and understand their own experiences.

thing. Instances of reclamation, for example, involve semantic shifts as well.⁸ When a term is hijacked, however, there is a profoundly negative social impact. Miranda Fricker (2007) describes the harm of being unable to understand and communicate one's experience caused by the lack of a social concept. Hermeneutic hijacking causes the loss of a social concept due to semantic shift.

We can thus think about hermeneutic hijacking as the opposite of reclamation. When a word is reclaimed, the term which once carried a pejorative meaning is repurposed into one that carries a positive meaning by the group it targets. If reclamation is wholly successful, then this leaves a linguistic gap. Those who wish to use the term to communicate something derogatory are left unable to do so. This linguistic gap is a positive thing. In cases of hermeneutic hijacking, there is also a semantic shift. This can come in degrees, but the literal meaning of the term is replaced by the meaning of the term when used nonliterally. This leaves a linguistic gap as well: one is unable to communicate (just) the literal meaning. In this case, though, the literal meaning is necessary for a particular social group, and the loss of the literal meaning does harm. The term is hijacked.

Take, for example, the term 'retarded'. The term started out as a diagnostic term intended to classify individuals with significant cognitive impairments. Call this the original usage. The term began to be used socially to pick out individuals loosely of the following description: 'is stupid for failing to do something that should be easy and obvious'. Call this the pejorative usage. Given the widespread adoption of the latter usage, it became the case that being picked out by the term was a bad thing. We therefore did

⁸ For work on reclamation, see: Robin Jeshion (2020); Claudia Bianchi (2014); Luvell Anderson (2018).

not have a way to appropriately classify individuals in the original group. In order to do so, we needed to create a new term ('intellectually disabled'). Although generally frowned upon in polite company, the use of 'retarded' today is used exclusively in the pejorative way; the word has lost its nonpejorative meaning. Although it may sound odd to say that we should still be using the term 'retarded' in the original way, this is because it was hijacked. It would have been better if it retained its use as a diagnostic term.

One might object here that hijacked terms can simply be replaced, renaming the diagnosis for which we now lack a term. However, there is little reason to suspect that the new term would not be co-opted in the same way. The fact that 'intellectually disabled' and 'special education' as pejoratives have followed psychiatry's move away from 'retarded' gives us reason to worry. As I will argue in the next section, MITs can serve as a touchstone for important aspects of one's identity. Renaming the terms is still destabilizing to those who identify with the original terms. When Asperger's Syndrome was removed from the DSM, many who had previously been diagnosed held on to the term because of the central role it played in how they understood themselves and their community. Renaming disorders when MITs are hijacked is an ineffective solution in which individuals with mental illnesses give up more than they get.

The term 'retarded' has been effectively hijacked. Current diagnostic terms are at serious risk of a similar fate. The nonliteral uses are more socially salient than the literal uses, sometimes to the extent that the literal meaning is missing entirely from widespread cultural awareness. For example, many people do not know what bipolar disorder is (how it is diagnosed, what the symptoms are, how it presents, etc.). However, the term 'bipolar' is often used socially to describe someone who is moody, indecisive, and irritable.

It is important to note that a term does not have to be entirely lost in order for the term to be effectively hijacked. It may be the case that mental health professionals are able to maintain a clear understanding of and communicate using MITs in conversations with other experts. As mentioned, hijacking comes in degrees. Sometimes (though definitely not always) experts and/or those to whom the (literal uses of the) terms apply retain a protected understanding of the term. This is still a loss as it limits the ability of those who understand the term to communicate effectively with laymen or otherwise the general public.⁹ Further, even in cases where clinicians are able to keep the literal and nonliteral meanings of MITs distinct, the terms may still be officially changed or removed in order to avoid plaguing patients with the stereotypes associated with the nonliteral meanings. Consider again the change from ‘retarded’ to ‘intellectually disabled’. I discuss this more in the following section.

It is helpful at this point to take a short detour and theorize the ways in which hermeneutic hijacking contributes to other forms of epistemic injustice faced by people who have been diagnosed with mental illnesses. I do not claim that hermeneutic hijacking accounts for all instances of epistemic injustice faced by those diagnosed, but it does seem likely that many of the ways that MITs are hijacked are related to the ways in which these other instances of epistemic injustice occur. As I said before, mental health professionals in clinical settings are the most likely to use MITs (even those in the process of being hijacked) in a literal way. But experts are not infallible and are themselves subject to biases. There is compelling evidence, for example, that many clinicians demonstrate bias against patients diagnosed with borderline personality disorder in ways consistent with

⁹ See Rebecca Mason (2011).

the nonliteral usage of ‘borderline’.¹⁰ Whether experts in these instances are using MITs literally or nonliterally, and whether the hijacking or the expert bias is the prior phenomenon, will likely depend on the particulars of the case. But this is a feature, not a bug, of my account of hermeneutic hijacking—part of the problem is the blurring of the line between literal and nonliteral uses, and so it is no surprise that hermeneutic hijacking will overlap with and reinforce other sorts of injustice.

MITs are particularly at risk of being hijacked by nonliteral uses for a few reasons.¹¹ The first is a general lack of understanding of the literal meaning of the terms. Many mental disorders remain largely misunderstood by or are unknown to laypeople. Because of this, there is nothing for people to draw from to differentiate nonliteral uses. Contrast this with terms like ‘cancer’ or ‘genius’, terms whose literal meanings are widely basically understood.¹² It is obvious that someone who forgets their wallet is not *literally* a genius, and therefore the term must be nonliteral when it is applied to them. Secondly, and relatedly, the nonliteral use of MITs is based on stereotypical behavior associated with the literal use. The behavioral dispositions which are taken to typify the actual disorders are the source of the nonliteral uses. Lastly, MITs are used nonliterally in an extremely consistent way. Contrast this with other terms, which can be used nonliterally in a wide variety of ways. The term ‘genius’, for example, can be used hyperbolically (to apply to someone who is very good at something) or sarcastically (to apply to someone who has made an obvious mistake), just to name a few. But MITs are not typically used in a variety of ways. Although some MITs are used pejoratively and some are used positively,

¹⁰ See, for example: Donald W. Black et al. (2011); Daniel Ring and Sharon Lawn (2025).

¹¹ Thank you to an anonymous reviewer for urging me to clarify what makes MITs vulnerable to hijacking, and for the helpful analogy with terms like ‘genius’.

¹² I provide a more thorough examination of nonliteral uses of illness terms like ‘cancer’ in Section 4.

particular MITs seem to be used in a very consistent way. The term ‘borderline’, for example, is always used nonliterally to mean roughly the same thing. These three features (a lack of understanding of the literal term, the close association between the literal and nonliteral uses, and the consistency of the nonliteral uses) all contribute to the particular vulnerability MITs have to being hijacked.

To summarize, hermeneutic hijacking involves a linguistic component and a social component. There is a semantic shift that leaves a linguistic gap, and this linguistic gap has negative consequences. I go into more detail about these negative consequences in the following section.

4. Harm

As mentioned in the previous section, hermeneutic hijacking results in a linguistic gap. When it comes to MITs, this gap is harmful in at least two ways. The first is a loss: we lose the ability to effectively communicate important information. The second is a burden: individuals who are mentally ill come to bear the burden of what is conveyed by the nonliteral uses. In order to fully appreciate the former, I will begin by explaining the latter.

What sorts of things do nonliteral uses of MITs imply about the target? Consider examples like the one in the previous section (using ‘bipolar’ to describe a moody coworker). These are pejorative nonliteral uses, which are used to convey something negative about the target. Terms used pejoratively are often used to insult, shame, or mock someone (sometimes oneself). As Robin Jeshion (2018) notes, however, the negative attitude expressed in pejoratives can take the form of pity (think, for example, of the term ‘cripple’). The pejorative nature of nonliteral uses of MITs can thus take many different forms. Commonly, MITs are used pejoratively to imply that the target is less trustworthy, is a bad person, or is unable to make rational decisions.

Other times, nonliteral uses of MITs will imply something positive about the target, usually that they have some special power that they can attribute to a purported personality quirk. Here are two examples:

OCD: Cheyenne arrives at Imogen's house and compliments Imogen on how tidy it is. Imogen, who does not meet the diagnostic criteria for obsessive-compulsive disorder, says, "Thanks, I am just OCD!"

ADHD: Luna sees a handful of TikToks by creators with ADHD discussing hyperfixation. Luna, though she knows that she does not have ADHD, relates to what the creators are saying, thinking of times when she has written 2,000 word essays in one day. When she and a peer are later discussing their grades on an assignment, Luna proudly proclaims, "I can just hyperfixate on my assignments for a few hours; I basically have ADHD."

In instances like these, MITs are being used to suggest that the target is a special sort of person, one who is better at or naturally inclined to succeed at something, in virtue of whatever it is about them that the term is meant to pick out.

It is conceivable that there are neutral nonliteral uses of MITs, though examples of this kind do not readily come to mind. One would have to do some work to show why an instance of supposed neutral nonliteral use of an MIT is not simply a false statement. My argument relies on the nonliteral uses being a prevalent way of using the term in a given linguistic community, and there do not seem to be any instances of MITs being used in a neutral nonliteral way that fit this description. Should such a usage become widespread, though, the argument would apply, so I leave the possibility open.

What is important about nonliteral uses of MITs is that the evaluative implications are not limited to the condition the term references; the MIT is used to convey something

positive or negative about the target's character, personality, or personhood. When someone complains about their "bipolar" coworker, it is the coworker, and not their mood swings, that is being denigrated. Further, the claim about the sort of person the target implicitly justifies certain treatment (avoidance, praise, exclusion, etc.). To state the point clearly, when used nonliterally, MITs are used to imply something about the sort of person someone is and seems to justify certain treatment of them.

This becomes a problem for those who have the mental illness that is being appropriated in this way. Because the MIT carries implications about the sort of person someone is and how they ought to be treated, someone diagnosed with bipolar disorder might reasonably worry that others will treat her as if she was 'bipolar' (pejorative). She might even mistakenly come to see herself as *being* 'bipolar' (pejorative), wrongly accepting the implications about who she is and how she ought to be treated in virtue of her diagnosis. Lastly, she will face extreme difficulty communicating that she is 'bipolar' (literal) and not 'bipolar' (pejorative) when part of the appropriate way to treat someone who is 'bipolar' (pejorative) is to dismiss what they say out of hand. She is effectively saddled with the evaluative baggage found in the nonliteral usage and is left without a way of contesting these implications.

It may be tempting to think that the baggage that comes along with positive nonliteral uses of MITs is, at worst, harmless, and, at best, beneficial. This is mistaken. The burden that comes with positive nonliteral uses of MITs is twofold: the minimization of the distress and dysfunction caused by the mental illness, and unreasonable expectations placed on the target. Lucienne Spencer and Havi Carel (2021: 2) describe the harm that comes with the wrongful depathologization of mental illnesses: "wrongful depathologization devalues the epistemic status of [mentally ill] people by reducing their

symptoms to mere personality traits, thus denying them a fully recognized psychiatric identity.” Those who have mental illnesses that have been wrongly depathologized experience both stigmatization—they are othered because of their illness—and the trivialization of their illness—they are thought to be exaggerating the difficulties they face because of their illness.

Spencer and Carel list four ways that positive stereotypes harm members of the stereotyped group:¹³

1. Positive stereotypes can compromise the well-being of members of their associated group as a result of an increased expectation to live up to the stereotype.
2. When a positively stereotyped individual fails to live up to these positive attributes, they may experience persecution.
3. Positive stereotypes emphasize group differences between marginalized and non-marginalized people.
4. Positive stereotypes draw attention to juxtaposing negative stereotypes attached to the marginalized group in question.

When a term is hijacked, a person is more than stereotyped. The term adopts the nonliteral meaning (at least conventionally), amplifying the harms detailed by Spencer and Carel.

Similar critiques have been made about positive but inaccurate portrayals of certain mental illnesses in media. A common example of this is the portrayal of the ‘autistic savant’ in movies like “Rain Man”. Although some might think that the ‘autistic

¹³ List reproduced in condensed form from Spencer and Carel (2021: 5-6).

savant' trope in media is positive because it shows that autistic individuals are capable of a high level of intelligence, the autistic community and disability scholars have pushed back on this idea, arguing that such portrayals contribute to widespread misinformation about the realities of autism and place unreasonable expectations on autistic individuals to be geniuses.¹⁴ Positive nonliteral uses of MITs make it hard for individuals with mental illnesses to accurately describe their experience and advocate for their unique needs.

One harm of hermeneutic hijacking in the case of MITs, then, is the baggage foisted upon those who are genuinely mentally ill. The other harm is the loss of the ability to use the terms literally. As I have said, MITs are used literally to convey diagnostic information, usually for clinical and research purposes. The ability to convey this information is essential for the health and well-being of individuals with mental illness.

I will not say too much about this harm (as the need to accurately identify medical conditions will seem obvious to many) but let me provide some support for my claim. Receiving a diagnosis of a mental illness should be the first step towards seeking and receiving effective treatment. However, because of the baggage carried by nonliteral uses of MITs, it is hard for practitioners to communicate to patients that their diagnosis does not mean what the patient is used to hearing the term mean. Patients may internalize the baggage associated with the common (nonliteral) use of the term, serving as a barrier to seeking treatment.¹⁵ Patients also experience discrimination from health care

¹⁴ For example, see Theodoto Ressa (2021).

¹⁵ For research on the relationship between self-stigma and treatment outcomes, see, for example: Nina Schnyder, Radoslaw Panczak, Nicola Groth, and Frauke Schultze-Lutter (2017); James D. Livingston and Jennifer E. Boyd (2010).

professionals, especially when they have been diagnosed with disorders whose names have been appropriated to mean ‘abusive’, such as borderline personality disorder.¹⁶

Lastly, diagnostic terms serve as important aspects of some people’s identity. In the same way that many cancer patients embrace labels like ‘cancer survivor’ to make sense of their own experience and find communities of similar people, MITs are valuable to many people diagnosed with mental illness. Losing those terms to their nonliteral uses steals, or at least taints, the ability to determine if and how much one’s mental illness fits into one’s own self-conception. This is well documented—many people who were once diagnosed with (or who would have met the diagnostic criteria for) what was once called Asperger’s Syndrome have expressed feeling a loss of identity and a challenge to their own self-understanding since the classification was removed in recent editions of the DSM.^{17,18} Of course, not everyone who receives a diagnosis will incorporate it into their identity (in the same way that not everyone who experiences cancer will see it as so personally important).¹⁹ Some diagnoses (like autism) may be more likely to be adopted as part of one’s identity than others (schizophrenia, for example), and individual people will ultimately differ with regard to how much they identify with their diagnosis or not. Even so, hermeneutic hijacking may play a role in whether or not someone does come to internalize a diagnosis. Some may be reluctant (consciously or not) to do so because they have internalized the evaluative baggage described above.

¹⁶ See, for example: Ciara Stiles, Rachel Batchelor, Andrew Gumley, and Ruchika Gajwani (2023); S. Veysey (2013); Ron B. Aviram, Beth S. Brodsky, and Barbara Stanley (2006).

¹⁷ C.f. Jennifer S. Singh (2011); Stephany Huynh, Adam McCrimmon, & Tom Strong (2020); Olivia Smith & Sandra C. Jones (2020).

¹⁸ The decision of those who have held onto the term ‘Asperger’s’ to describe their identity, as well as those who have embraced the label ‘aspie’, may be said to be engaging in what Trystan S. Goetze (2017) calls hermeneutical dissent: the creation and use of language and concepts by members of marginalized groups despite a lack of hermeneutic resources in the broader (dominant) community.

¹⁹ Thank you to an anonymous reviewer for pushing me on this point.

It is important to note that a person can be harmed in this way without realizing it. The consequences of being perceived as bipolar (pejorative) instead of bipolar (literal) may be subtle, especially to someone who may have internalized that their condition means there is something bad about them. It is also not the case that everyone will experience the effects of this hijacking to the same extent. As with many social harms, those who are already marginalized will face a disproportionate amount of prejudice. There will be people who claim to not be harmed by the process as I have described it, but this is not good reason to doubt the prevalence or severity of the harm in a generalized way.

When MITs are hijacked, there are two consequences, both of which disproportionately affect individuals with mental illnesses: the baggage carried in nonliteral uses of MITs is foisted upon people who do not deserve it, and we lose the ability to use the terms as originally intended. I have limited my argument so far to terms that are currently listed in the DSM-5 as either conditions or symptoms of conditions. There are lots of terms, however, that are not in the DSM-5 that nevertheless seem to reference mental illness (for example, ‘crazy’). In the next section, I consider how my account might extend to other terms, starting with these “looser” MITs.

5. Expanding the Account

I take there to be three phenomena that my account potentially expands to: the nonliteral use of terms that were in former editions of the DSM (“hysterical”), metaphors that gesture at mental illness without using specific diagnostic terms (“go take your meds”), and the wide variety of nonliteral speech about things that have nothing to do with mental illness. I consider each in turn.

The first category includes terms like “hysterical”, “Asperger’s”, “multiple personality disorder”, etc. Whether my account extends to these terms will depend on a variety of factors, including how long it has been since the term has been used literally and whether the baggage carried by nonliteral uses latch on to distinct groups of marginalized people. As I mentioned in the previous section, many people use “Asperger’s” in the way it was originally intended to be used and claim it as an important part of their identity despite it no longer being an officially recognized classification in the DSM. To use the term nonliterally to mean something distinct from this original meaning would thus have the same effect as any other MIT that remains in the DSM. Other terms, like “hysterical”, are so divorced at this point from their original usage that it is more accurate to say that they have already been hijacked. Though they might carry the harms associated with other types of speech (in the case of “hysterical”, other pejorative speech), it is not (the risk of) hermeneutic hijacking that explains this harm.

As for the second category, I am thinking of instances in which the speaker gestures at something relevant to mental health in a nonliteral way to make a point. Just like with MITs, this can be pejorative (“he needs to be committed”) or positive (“hiking is my therapy”). I am unsure if it is correct to say that these terms and phrases are (at risk of) being hijacked *per se*, due to the more nebulous nature of their definitions, but something akin to it is happening. Consider “hiking is my therapy.” The speaker does not have to intend to say that hiking is as beneficial for their mental health as clinical therapy for this phrase to be literally true; the term “therapy” is not so strictly defined. Many people, however, do intend to exaggerate the effect of hiking on their wellbeing, using the term “therapy” nonliterally even though it is (perhaps) true. Although the term “therapy” doesn’t take on a *new* meaning as the nonliteral use becomes conventionalized, the

boundary between the two meanings (clinical therapy vs. therapy broadly construed) becomes blurred. As anyone who has been asked “Have you tried hiking?” in response to sharing a mental illness diagnosis will attest, this blurring has a very similar effect to what I describe in Section 3.

Lastly, it is worth considering whether anything unrelated to mental health is at risk of hijacking. I think it would be utterly bizarre for the phenomenon to be limited solely to MITs and topics that are closely related, so the short answer is surely yes. I have focused on MITs because they are a clear case, but we can determine what it is about MITs that make them so vulnerable in order to say how the account might generalize. MITs are prone to hijacking because their literal use is not solidly established in most contexts, because many people are simultaneously dubious of experts and confident in their own intuitions, and because the particular way that MITs are used nonliterally make it hard for those usages to be challenged. What other topics might be similar?

An obvious answer is other illness and health terms. Susan Sontag (1977) provides a striking account of the way metaphors about illness, tuberculosis and cancer in particular, impact those who are ill. She provides a genealogy of illness terms similar to the one I describe above: there is a disease (often one that is already stigmatized), the name of that disease is used as a metaphor to convey some moral or normative message, the name of the disease begins to encode the baggage associated with the nonliteral usage, and ultimately those with the disease have no escape from this baggage. When health or disease terms and their nonliteral uses fit this pattern, they surely will fit my account.

Not all health metaphors will fit this pattern, though. Awareness campaigns have done wonders to both increase knowledge about and minimize stigma towards many illnesses. For example, I do not take “He is a cancer on our society” to be a problematic

use of the term “cancer” for a few reasons. The first is that the vast majority of people nowadays have a solid understanding of (at least the basics about) cancer, so the term’s meaning has a more solid foundation to stand upon. The second is that the badness rooted in the term “cancer” does not seem to extend to people with cancer in the same way that the badness of “bipolar” does.²⁰ In other words, we seem capable of talking about the badness of cancer without implying anything at all about the character of the cancer patient, but our ability to distinguish between the badness of “bipolar” and “person with bipolar” is unreliable at best.

I do not have the space to explore all of the potential candidates for my account, but it seems that any term or phrase (1) whose literal or intended meaning is not firmly grasped by a large percentage of people and (2) that we have strong reasons for protecting that literal or intended meaning from dilution or appropriation is at risk for hermeneutic hijacking. Hermeneutic hijacking may explain the resistance to use the term “Holocaust” without great care—we need to be able to maintain the ability to use the word to describe only the greatest of atrocities.

6. Conclusion

In *Illness as Metaphor*, Susan Sontag (1977: 56) identifies her main point “that illness is *not* a metaphor, and that the most truthful way of regarding illness—and the healthiest way of being ill—is one most purified of, most resistant to, metaphoric thinking.” I do not know if such a hard line is necessary, but certainly there is a line to be drawn. Not all

²⁰ Interestingly, this does not seem true about some forms of cancer and other diseases (specifically, those that we take to be caused the actions of the person who is sick). Some of these illness terms are probably used in the same way I have described above. Such usages, however, are simply less common than non-literal uses of MITs—perhaps it is just less poetic to metaphorically say that your disliked coworker has cirrhosis.

terms are apt as metaphorical fodder. In this paper, I have provided one reason to not use mental illness terms metaphorically: doing so contributes to the hermeneutic hijacking of the term, leaving a linguistic gap and contributing to stigma faced by the mentally ill.

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